HOSPITAL CLINICAL TRANSFER DOSSIER

CONFIDENTIAL MEDICAL RECORD • SBAR SUMMARY & VERIFIED HANDOFF

PATIENT: Eleanor Vance	**MRN:** 883921	**DOB / AGE:** 1958-04-12 (68y F)
FROM: Medical ICU (Bed 14)	**TO:** Step-Down Unit (Bed 202)	**ATTENDING:** Dr. Robert Chen, MD
ADMIT DATE: 2026-08-10	**CODE STATUS: FULL CODE**	**TRANSFER PACKET:** S2 FIXED DOSSIER

1. SBAR CLINICAL HANDOFF NARRATIVE (PROVENANCE-GROUNDED)

SITUATION:
68yo female recovering from acute hypoxemic respiratory failure secondary to severe community-acquired pneumonia [transfer_summary.pdf:p1].
Citations: none recorded for this section.

BACKGROUND & PAST MEDICAL HISTORY:
Admitted 5 days ago to MICU, intubated x 3 days, successfully extubated 48h ago [admission_hnp.pdf:p1]. PMHx: COPD, Type 2 DM, HTN.
Citations: none recorded for this section.

CLINICAL ASSESSMENT & CURRENT TRAJECTORY:
Afebrile, SpO2 96% on 2L nasal cannula. Lungs clear with faint bibasilar crackles. Alert and oriented x 4 [icu_progress_day5.pdf:p2].
Citations: none recorded for this section.

RECOMMENDATIONS & STEP-DOWN PLAN:
Transfer to Step-Down unit. Wean O2 as tolerated. Complete 48h IV Ceftriaxone then switch to oral Cefpodoxime [provider_orders.pdf:p1].
Citations: none recorded for this section.

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2. SAFETY VERIFICATION GATEWAY (MANDATORY HUMAN SIGN-OFF)

ALLERGY / ADVERSE DRUG REACTION	REACTION SEVERITY	VERIFICATION STATUS
Penicillin (Anaphylaxis)	Severe / Anaphylaxis Risk	✓ **CONFIRMED: Audit RN (RN-824)**
Sulfa Drugs (Rash)	Moderate Sensitivity	✓ **CONFIRMED: Audit RN (RN-824)**

RESUSCITATION CODE STATUS VERIFICATION:

RECORDED CODE STATUS: FULL CODE	✓ **SIGNED OFF BY: Audit RN (RN-824)**

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3. CONSERVATIVE MEDICATION RECONCILIATION (100% RECALL)

MEDICATION & STRENGTH	DOSE & ROUTE	FREQUENCY	SAFETY / RECON WARNING	STATUS
Heparin Sodium Infusion heparin	18 units/kg/hr IV IV Continuous	Continuous	*No Conflict*	**✓ HIGH ALERT (Verified)**
Insulin Glargine (Lantus) insulin glargine	20 units SubQ Subcutaneous	Nightly at 21:00	*No Conflict*	**✓ HIGH ALERT (Verified)**
Ceftriaxone Sodium ceftriaxone	1g IV Piggyback Intravenous	Every 24 hours	*No Conflict*	**✓ Standard**
Cefpodoxime Proxetil cefpodoxime	200mg Oral Oral	Every 12 hours	**THERAPEUTIC DUPLICATION (Cephalosporin Antibiotic): Concurrent class order with Ceftriaxone Sodium (active_mar.txt)**	**✓ Standard**

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4. ACTIVE PHYSICIAN ORDERS & PENDING DIAGNOSTICS

No active physician orders recorded in the ingested source documents. Verify directly with the sending unit before transfer.

PENDING LABORATORY / DIAGNOSTIC STUDIES:

No pending laboratory or diagnostic studies recorded in the ingested source documents.

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5. MOBILITY STATUS, FALL RISK & ISOLATION PRECAUTIONS

No mobility, fall-risk, or isolation assessments recorded in the ingested source documents. Complete the assessments with the sending unit before transfer.

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APPENDIX 1: PRIMARY CLINICAL SOURCE RECORD (PAGE 6/10)

DOCUMENT: Certified ACTIVE_MAR

HOSPITAL OF THE UNIVERSITY MEDICAL CENTER
ELECTRONIC MEDICATION ADMINISTRATION RECORD (eMAR)
PATIENT: Eleanor Vance | MRN: 883921 | GENERATED: 2026-08-15 09:30 EST
ACTIVE MEDICATION ORDERS & INFUSIONS:
1. HEPARIN SODIUM INFUSION [HIGH-ALERT MEDICATION: ANTICOAGULANT]
- Dose/Rate: 18 units/kg/hr IV Continuous Infusion (Current Rate: 1,250 units/hr)
- Indication: Acute DVT Prophylaxis in high-risk ICU bedrest
- Route: Intravenous Continuous
- Last Dose Verified: 2026-08-15 08:00 EST (Anti-Xa Therapeutic: 0.42 IU/mL)
- Nursing Protocol: Dual-RN independent double check mandatory for rate titration and bag change.
2. INSULIN GLARGINE (LANTUS) [HIGH-ALERT MEDICATION: INSULIN ANALOGUE]
- Dose: 20 units SubQ
- Schedule: Nightly at 21:00 EST
- Indication: Basal management of Type 2 Diabetes Mellitus
- Route: Subcutaneous
- Last Dose Given: 2026-08-14 21:00 EST

CERTIFIED ATTESTATION: *Verified authentic unredacted clinical record for inter-unit patient handoff.*

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APPENDIX 2: PRIMARY CLINICAL SOURCE RECORD (PAGE 7/10)

DOCUMENT: Certified ADMISSION_HNP

HOSPITAL OF THE UNIVERSITY MEDICAL CENTER
ADMISSION HISTORY & PHYSICAL (H&P;)
PATIENT: Eleanor Vance | MRN: 883921 | DOB: 1958-04-12 | AGE: 68 | GENDER: F
ADMISSION DATE: 2026-08-10 03:45 EST | ATTENDING: Dr. Robert Chen, MD | UNIT: Medical ICU Bed 14
CHIEF COMPLAINT:
Acute respiratory failure, shortness of breath, high fevers.
HISTORY OF PRESENT ILLNESS:
68-year-old female with a history of severe COPD (GOLD Stage III), Type 2 Diabetes Mellitus, and Hypertension who presented to the Emergency Department with a 4-day history of productive cough, pleuritic chest pain, and worsening dyspnea. In the ED, patient was in severe respiratory distress with SpO2 81% on room air, RR 34/min, and arterial blood gas showing acute hypoxemic and hypercapnic respiratory failure (pH 7.24, pCO2 64, pO2 52). Chest X-ray demonstrated dense right middle and lower lobe consolidation consistent with severe community-acquired pneumonia. Patient was intubated in the ED and transferred directly to the Medical Intensive Care Unit (MICU).
PAST MEDICAL HISTORY:
1. Chronic Obstructive Pulmonary Disease (COPD) - on home tiotropium and albuterol PRN
2. Type 2 Diabetes Mellitus - on insulin glargine
3. Essential Hypertension
4. Osteoarthritis
ALLERGIES:
1. PENICILLIN - SEVERE ANAPHYLAXIS (Laryngeal edema, hives, hypotension in 2018)
2. SULFA DRUGS - RASH / URTICARIA

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APPENDIX 3: PRIMARY CLINICAL SOURCE RECORD (PAGE 8/10)

DOCUMENT: Certified ICU_PROGRESS_DAY5

HOSPITAL OF THE UNIVERSITY MEDICAL CENTER
MICU DAILY CRITICAL CARE PROGRESS NOTE (DAY 5)
PATIENT: Eleanor Vance | MRN: 883921 | DATE: 2026-08-15 08:00 EST | UNIT: MICU Bed 14
SUBJECTIVE / 24-HOUR EVENTS:
Patient is now Hospital Day 5, Post-Extubation Day 2. Patient was successfully extubated 48 hours ago following a 3-day course of mechanical ventilation. She has tolerated weaning without stridor, respiratory fatigue, or significant tachypnea. Slept well overnight. Mild productive cough with clear secretions. Denies chest pain or palpitations.
OBJECTIVE / PHYSICAL EXAM:
- Vitals: T 98.4 F (Afebrile), HR 78 bpm regular, BP 128/74 mmHg, RR 18/min, SpO2 96% on 2L nasal cannula.
- General: Alert, oriented x 4, sitting upright in bed in no acute distress.
- Lungs: Clear to auscultation bilaterally with faint bibasilar crackles; no wheezing or rhonchi. Good respiratory effort.
- Cardiovascular: RRR, S1/S2 present, no murmurs, gallops, or friction rubs. 2+ peripheral pulses.
- Abdomen: Soft, non-distended, non-tender, active bowel sounds.
- Extremities: Warm, dry. Mild trace pedal edema bilaterally. Calf non-tender.
- Neuro: Cranial nerves II-XII grossly intact. Strength 4/5 in lower extremities bilaterally secondary to ICU deconditioning.
- Morse Fall Risk Assessment: Score 55 (High Risk: IV access, history of bedrest, secondary diagnosis).
- Isolation Precautions: Standard Precautions (Droplet precautions discontinued following 3 negative viral PCR panels).
ASSESSMENT & CLINICAL TRAJECTORY:

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APPENDIX 4: PRIMARY CLINICAL SOURCE RECORD (PAGE 9/10)

DOCUMENT: Certified LAB_DIAGNOSTICS

HOSPITAL OF THE UNIVERSITY MEDICAL CENTER
CLINICAL LABORATORY & DIAGNOSTIC REPORT
PATIENT: Eleanor Vance | MRN: 883921 | DATE: 2026-08-15 06:30 EST
1. COMPLETE BLOOD COUNT (CBC):
- WBC: 9.4 x10^3/uL (Normal, down from 18.2 on admission)
- Hemoglobin: 11.8 g/dL (Stable)
- Hematocrit: 35.4%
- Platelets: 245 x10^3/uL (Normal)
2. BASIC METABOLIC PANEL (BMP):
- Sodium: 138 mEq/L
- Potassium: 4.1 mEq/L
- Chloride: 102 mEq/L
- CO2 (Bicarbonate): 28 mEq/L (Compensated respiratory acidosis)
- BUN: 18 mg/dL
- Creatinine: 0.88 mg/dL (Normal eGFR >60 mL/min)
- Blood Glucose: 134 mg/dL (Controlled)

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APPENDIX 5: PRIMARY CLINICAL SOURCE RECORD (PAGE 10/10)

DOCUMENT: Certified PROVIDER_ORDERS

HOSPITAL OF THE UNIVERSITY MEDICAL CENTER
PHYSICIAN TRANSFER & STEP-DOWN ORDERS
PATIENT: Eleanor Vance | MRN: 883921 | ORDER DATE: 2026-08-15 09:00 EST | ORDERING PROVIDER: Dr. Robert Chen, MD
TRANSFER DIRECTIVES:
1. Transfer patient from Medical ICU to Step-Down Intermediate Care Unit (Bed 202).
2. Code Status: FULL CODE (Confirmed).
3. Activity: Assist of 1 with walker for all transfers and ambulation (Morse Fall Risk: 55).
4. Diet: Diabetic Regular Diet (1,800 kcal/day).
5. Respiratory: Continuous pulse oximetry. Titrate nasal cannula O2 to maintain SpO2 92-96%.
6. Antibiotic Plan: Complete 48 hours of IV Ceftriaxone, then transition to Oral Cefpodoxime 200mg BID to complete a 10-day total course. Do NOT administer both concurrently.
7. Anticoagulation: Continue IV Heparin infusion per protocol; transition to SubQ enoxaparin once ambulating >100 feet.
8. Follow-up Labs: Morning BMP, CBC with diff, and Repeat Chest X-Ray in 48 hours.

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TAMPER-EVIDENT AUDIT TRAIL & NURSE SIGN-OFF LEDGER

AUDIT ITEM	RECORD VALUE / TIMESTAMP
Packet Cryptographic Digest (SHA-256)	`46d635d3f3a518b0d79b4035b27f75ff3b17e5d3501cb71b779c58f9a5b9edf9`
Allergies Gate Verification	Audit RN (RN-824) @ 2026-08-24T15:16:38.382434+00:00
Code Status Gate Verification	Audit RN (RN-824) @ 2026-08-24T15:16:38.412329+00:00
High-Risk Meds Dual Verification	RN 1: Audit RN (RN-824) RN 2: RN Mark Taylor (RN-5104)
Transfer Dossier Total Page Count	10 Certified Clinical Pages + 1 Audit Appendix